How to Read This Guide

Pages 4–6 of this handout contain **purine heat maps** — colour-coded food tables that show exactly how much purine is in each Filipino food item and whether it is safe, limited, or avoided on a gout-friendly diet. This page explains how to interpret them.

WHAT ARE PURINES — AND WHY DO THEY MATTER?

Purines are natural compounds found in many foods. When your body breaks them down, the end product is **uric acid**. If uric acid builds up faster than your kidneys can excrete it, it crystallises in joints and soft tissue — causing the painful swelling of a gout flare. In CKD, the kidneys are already less efficient at clearing uric acid, so dietary management becomes especially important.

THE COLOUR-CODING SYSTEM — PAGES 4–6

Colour	Risk level	Purine range	What it means for you
RED	Avoid	>300 mg / 100 g	Skip entirely or eat only once per month at most. These push uric acid significantly above target.
ORANGE	Strictly limit	150–300 mg / 100 g	Maximum 1–2 × per week, small portion (½ cup or less). Combine with high-fluid intake that day.
AMBER	Moderate caution	80–150 mg / 100 g	Up to 3–4 × per week in normal portions. Monitor your uric acid if eating regularly.
GREEN	Safe — eat freely	<80 mg / 100 g	No purine restriction. Form the foundation of every meal. These are your go-to foods.
BLUE	Alcohol — avoid	Variable	Beer, brandy, and distilled spirits block uric acid excretion regardless of purine content. See page 5.

HOW TO READ THE TABLE COLUMNS

Column	What it shows	How to use it
Food Item (Filipino / English)	The Filipino common name comes first so you can identify it at the market or table.	Match the food you are about to eat.
Purines mg / serving	Purine content in a typical Filipino single serving (e.g., 1 cup, 1 piece, 100 g).	Add up your servings across the day. Stay below 400 mg total.
Purines mg / 100 g	A standardised reference weight — lets you compare foods fairly regardless of portion.	Useful when buying packaged foods or adjusting portion size.
Risk	A one-word summary of the colour category (Avoid / High / Moderate / Low / Safe).	Quick scan without looking at numbers.

YOUR DAILY PURINE BUDGET

Patient type	Daily purine target	Protein target	Note
General adult (no gout / CKD)	< 600 mg/day	0.8 g/kg/day	No restriction; avoid excess organ meat and shellfish.
Hyperuricaemia without gout	< 500 mg/day	0.8–1.0 g/kg/day	Reduce if UA persistently > 8 mg/dL.
Established gout (not on medication)	< 400 mg/day	0.8 g/kg/day	Diet alone lowers UA by ~1–2 mg/dL.
Gout on febuxostat / allopurinol	< 500 mg/day	0.8 g/kg/day	Medication does the heavy lifting; diet prevents spikes.
CKD Stage 1–3 with gout	< 400 mg/day	0.8 g/kg/day	Monitor K■ — some low-purine foods are high-potassium.
CKD Stage 4–5 / Dialysis with gout	< 400 mg/day	Per nephrologist	Protein and potassium restrictions may override purine targets.

■ Diet alone typically reduces serum uric acid by 1–2 mg/dL. If your target is < 6 mg/dL (or < 5 mg/dL in CKD), medication (febuxostat or allopurinol) is almost always required alongside dietary changes — see page 6 for the medication reference.

Uric Acid Targets & Medication Reference

Use this page alongside the food tables on pages 4–6. Diet controls purine load; medication controls uric acid synthesis and excretion. Both are usually needed to reach target in patients with established gout or CKD.

SERUM URIC ACID TARGETS — TREAT-TO-TARGET APPROACH

Patient Group	Target UA	Rationale	Monitoring
No gout (prevention)	< 7.0 mg/dL (men) < 6.0 mg/dL (women)	Saturation point of urate at body temperature.	Annual UA with metabolic panel.
Established gout (no tophi)	< 6.0 mg/dL	Below this level, monosodium urate crystals dissolve over months.	UA every 2–4 weeks until target, then every 6 months.
Gout with tophi or frequent flares	< 5.0 mg/dL	Lower target accelerates tophus resolution.	UA every 4 weeks until target, then every 6 months.
CKD Stage 1–3 with gout	< 6.0 mg/dL	Standard target; renal dosing for medications applies.	UA + eGFR + creatinine every 3 months.
CKD Stage 4–5 / Dialysis with gout	< 6.0 mg/dL (if tolerated)	Febuxostat preferred over allopurinol in severe CKD.	Discuss target and monitoring frequency with your nephrologist.
Asymptomatic hyperuricaemia (UA > 9 mg/dL)	Consider < 6.0 mg/dL	KDIGO 2024 recommends treatment if UA > 9 mg/dL with CKD progression.	Reassess every 6 months.

MEDICATIONS FOR GOUT — CKD DOSING & NOTES

Medication	Purpose	Usual dose	CKD notes	Local notes (PH)
Febuxostat (Feburic®)	Urate-lowering (XO inhibitor)	40–80 mg once daily	No dose adjustment needed until eGFR < 15. Preferred over allopurinol in CKD Stage 3b–5.	Covered by PhilHealth Z-Benefit for gout with CKD. Available branded and generic.
Allopurinol	Urate-lowering (XO inhibitor)	100 mg/day; titrate by 100 mg q4wks to target	Reduce in CKD: eGFR 10–50 → 100–200 mg/day; eGFR < 10 → 100 mg every 2 days.	Cheapest option. Risk of DRESS syndrome — stop immediately if rash appears.
Colchicine (acute flare)	Anti-inflammatory (flare prevention)	0.5–0.6 mg twice daily for prophylaxis; 1.2 mg then 0.6 mg 1 hr later for acute flare	Reduce to once daily if eGFR 30–60. Avoid if eGFR < 10 or on dialysis.	Widely available. Do NOT combine with clarithromycin or cyclosporine — toxicity risk.
Naproxen / Mefenamic acid (acute flare only)	NSAID — anti-inflammatory	Short course only (3–5 days)	AVOID in CKD Stage 3b and above — accelerates kidney decline and raises BP.	Common OTC in Philippines — patients often self-medicate. Counsel against this.
Prednisone / Methylprednisolone (acute flare)	Corticosteroid — anti-inflammatory	30–40 mg/day × 3–5 days, taper	Safe in CKD; watch blood sugar in diabetics.	Used when colchicine and NSAIDs are contraindicated.
Probenecid	Uricosuric — increases UA excretion	500 mg twice daily	Contraindicated if eGFR < 30.	Rarely used in Filipino CKD patients. Ensure high fluid intake (> 2 L/day).

WHEN TO CONTACT YOUR DOCTOR OR GO TO THE ER

Situation	Action
Sudden severe joint pain, swelling, warmth (especially big toe, ankle, knee)	Take colchicine as prescribed. If no relief in 24 hrs, call clinic.
Skin rash within 3 months of starting allopurinol	STOP allopurinol immediately. Go to ER — possible DRESS syndrome.
BP > 180/110 mmHg during a flare	Go to ER. NSAIDs worsen hypertension and kidney function.

Joint swelling with fever > 38.5°C	Go to ER — rule out septic arthritis (an emergency, not a gout flare).
UA > 10 mg/dL on repeat testing despite medication	Schedule a consultation. Dose titration or medication switch needed.

Building Your Gout-Friendly Filipino Meal

The food tables on pages 4–5 tell you what each food scores. This section shows you **how to put a full day of Filipino eating together** — which patterns protect you, which patterns trigger flares, and how to alkalise your urine to help clear uric acid faster.

DAILY MEAL TEMPLATE

BREAKFAST

- Sinangag (garlic fried rice) — 1 cup
- Itlog (egg, scrambled or boiled) — 1–2 pcs
- Toyo-calamansi dip (small amount)
- Kamatis / ensaladang talong
- 1 glass of water or low-fat milk

MORNING SNACK

- Banana or papaya — 1 pc
- Low-fat yogurt — 1 small cup
- 1–2 glasses of water

LUNCH

- Rice — 1 cup (plain)
- Lean chicken adobo (skinless) — 1 pc
- Pinakbet or ginisang gulay — 1 cup
- Sabaw (broth) — 1 bowl
- 2 glasses of water

AFTERNOON SNACK

- Kamote or saging na saba — 1 pc
- Water or fresh buko juice — 1 glass

DINNER

- Rice — 1 cup
- Tilapia / bangus (steamed/grilled) — 1 slice
- Monggo gulay — ½ cup (amber; balance with vegetables)
- Sayote or upo soup — 1 bowl
- 2 glasses of water

HIDDEN TRIGGER PATTERNS

THE PIKA-PIKA TRAP

Pulutan sessions with beer, bagooong, dilis, tokwa, mani at gabi — every item on the table is high-purine, and alcohol blocks uric acid excretion. A single pika-pika session can raise UA by 2–3 mg/dL the next morning.

SINIGANG NA BABOY / BULALO

The bone broth concentrates purines released from pork ribs and marrow. Switch to sinigang na isda (tilapia) or sinigang na manok and drink the sabaw sparingly.

CANNED SARDINES / TUYO DAILY

Extremely high purine content (>300 mg/100 g). Many patients eat this daily as a cheap protein source — this alone can prevent reaching UA target. See page 4 for alternatives.

SOFTDRINKS DURING FLARES

Fructose in softdrinks stimulates uric acid production independently of purines. Avoid all sugar-sweetened beverages, especially during and after a flare.

ORGAN MEAT (GOTO, DINUGUAN, PAPAITAN)

Liver, kidneys, brain, and intestines have the highest purine density of any food group. Even small portions in a stew significantly raise the purine load.

FLUID & ALKALINE TIPS

Target 2.5–3 L of fluids per day

Dilutes uric acid in blood and urine. Water and buko juice are best. Avoid softdrinks and beer.

Alkaline foods at every meal

Kamatis, pechay, sayote, upo, malunggay, calamansi juice — these raise urine pH and increase urate solubility. See page 5, vegetable section.

Vitamin C 500 mg daily (with physician approval)

Mild uricosuric effect (~0.5 mg/dL reduction). Found in calamansi, guava, and papaya. Supplement only if diet is insufficient.

Limit coffee to 2 cups/day

Moderate coffee consumption is associated with lower UA. Avoid 3-in-1 mixes — excess sugar offsets the benefit.

■ This guide is for general patient education. Individual dietary advice depends on your eGFR, serum potassium, and medication regimen. Consult your physician before making major dietary changes — some high-potassium vegetables safe for gout may need limiting in CKD Stage 4–5.

GOUT & URIC ACID IN CKD — HIGH-PURINE FILIPINO FOODS — AVOID OR STRICTLY LIMIT

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DAILY PURINE TARGET: <400 mg/day (gout) | <200 mg/day (severe or tophaceous gout)

Plant-based purines (vegetables, beans) DO NOT raise flare risk — animal-derived purines do.

Source: FNRI-Philippines, USDA, ACR 2020, KDIGO 2024. Values are approximate per typical Filipino serving.

Food Item (Filipino / English)	Serving	Purines	mg/day target	Risk
ORGAN MEATS (LAMAN-LOOB) — single biggest risk category; avoid entirely				
Sweetbreads (thymus/pancreas)	100g	**~825**	206%	EXTREME
Atay ng baka / beef liver	100g	**~550**	138%	EXTREME
Bato ng baka / beef kidney	100g	**~510**	127%	EXTREME
Dinuguan (pork blood stew)	1 cup	**~480**	120%	EXTREME
Atay ng manok / chicken liver	100g	**~470**	118%	EXTREME
Goto / tripe (callos)	100g	**~470**	118%	EXTREME
Utak / brain	100g	**~360**	90%	EXTREME
Kasim / balun-balunan / gizzard	100g	**~280**	70%	EXTREME
Isaw ng manok / chicken intestines	2 sticks (~50g)	**~250**	62%	EXTREME
Bituka ng baboy / pork intestines	100g	**~230**	57%	VERY HIGH
Betamax (pork/chicken blood)	1 stick (~50g)	**~190**	48%	VERY HIGH
Puso ng manok / chicken heart	100g	**~170**	42%	HIGH
SEAFOOD & FISH — small fish, shellfish, dried/fermented = highest risk				
Sardinas (canned, with bones)	1/2 can (100g)	**~480**	120%	EXTREME
Dilis / dried anchovies (bilad)	30g	**~340**	85%	EXTREME
Tuyo / dried salted fish	30g	**~290**	72%	EXTREME
Bagoong alamang / shrimp paste	1 tbsp (15g)	**~250**	62%	EXTREME
Tahong / green mussels	100g	**~290**	72%	EXTREME
Talaba / oysters	100g	**~290**	72%	EXTREME
Tuna (fresh, grilled)	100g	**~290**	72%	EXTREME
Hipon / sugpo / shrimp, prawns	100g	**~265**	66%	VERY HIGH
Galunggong / mackerel scad	100g	**~250**	62%	VERY HIGH
Bagoong isda / fermented fish paste	1 tbsp (15g)	**~220**	55%	VERY HIGH
Hibe / dried small shrimp	1 tbsp (10g)	**~200**	50%	VERY HIGH
Pusit / squid	100g	**~190**	48%	HIGH
Alimasag / alimango / crab	100g	**~150**	38%	HIGH
Bangus / milkfish (fresh, grilled)	100g	**~165**	41%	HIGH
Tilapia (grilled)	100g	**~120**	30%	MODERATE
Cream dory / pangasius	100g	**~95**	24%	MODERATE
MEAT, POULTRY & PROCESSED MEATS				
Lechon kawali / liempo (pork belly)	100g	**~145**	36%	HIGH
Adobong manok / chicken adobo	100g	**~140**	35%	HIGH
Beef brisket / tapa / karne	100g	**~135**	34%	HIGH
Chicken breast, boiled (no skin)	100g	**~135**	34%	HIGH
Tocino / longganisa / hotdog	100g	**~110**	28%	HIGH
Spam / corned beef (canned)	100g	**~110**	28%	HIGH
Pork shoulder / kasim (inihaw)	100g	**~130**	32%	HIGH

GOUT & URIC ACID IN CKD — SAFE & MODERATE PURINE FOODS + COMPLETE MEAL GUID

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GREEN (SAFE) < 50 mg/serving | TEAL (LOW) 50–100 mg | AMBER (MODERATE) 100–200 mg

Plant-based purines do NOT raise flare risk in modern studies. Prioritize eliminating organ meats, shellfish, beer, and HFCS drinks.

Food Item (Filipino / English)	Serving	Purines	mg/day target	Risk
VEGETABLES & LEGUMES — safe despite measurable purines (plant purines ≠ flare risk)				
Upo / bottle gourd	1 cup cooked	**~5**	1%	SAFE
Sayote / chayote	1 cup cooked	**~10**	2%	SAFE
Repolyo / cabbage	1 cup cooked	**~12**	3%	SAFE
Talong / eggplant	1 cup cooked	**~14**	4%	SAFE
Labanos / white radish	1 cup cooked	**~15**	4%	SAFE
Kamatis / tomato	1 medium	**~15**	4%	SAFE
Pipino / cucumber	1 cup	**~10**	2%	SAFE
Kangkong (boiled, water discarded)	1 cup cooked	**~40**	10%	SAFE
Pechay / bok choy	1 cup cooked	**~30**	8%	SAFE
Ampalaya / bitter gourd	1 cup cooked	**~35**	9%	SAFE
Kalabasa / squash	1 cup cooked	**~75**	19%	SAFE
Asparagus	1 cup cooked	**~75**	19%	SAFE
Kabute / mushroom (fresh)	1 cup cooked	**~90**	22%	SAFE
Spinach / alugbati	1 cup cooked	**~60**	15%	SAFE
Malunggay / moringa	1/2 cup cooked	**~55**	14%	SAFE
Munggo / mung beans (cooked)	1 cup	**~135**	34%	MODERATE
Garbanzos / chickpeas (cooked)	1/2 cup	**~85**	21%	SAFE
Tokwa / tofu (firm)	100g	**~70**	18%	SAFE
GRAINS, EGGS, DAIRY & FRUITS — foundation of a gout-friendly diet				
Kanin / white rice (1 cup)	1 cup cooked	**~10**	2%	SAFE
Pandesal / white bread	2 pieces	**~12**	3%	SAFE
Bihon / rice noodles	1 cup cooked	**~10**	2%	SAFE
Pasta / macaroni (cooked)	1 cup cooked	**~15**	4%	SAFE
Mais / corn (boiled)	1 ear	**~45**	11%	SAFE
Itlog / whole egg	1 large	**~5**	1%	SAFE
Puti ng itlog / egg white	2 whites	**~2**	0%	SAFE
Gatas / low-fat milk	1 cup (240mL)	**~15**	4%	SAFE
Yogurt (plain, low-fat)	1 cup	**~20**	5%	SAFE
Mansanas / apple	1 medium	**~9**	2%	SAFE
Peras / pear	1 medium	**~8**	2%	SAFE
Papaya (ripe)	1 cup	**~8**	2%	SAFE
Saging / banana	1 medium	**~8**	2%	SAFE
Seresa / cherries (fresh)	10–12 pieces	**~5**	1%	PROTECTIVE
Calamansi (fruit)	3–5 pieces	**~4**	1%	PROTECTIVE
BEVERAGES — beer and HFCS drinks are the biggest hidden drivers				
Beer (any / lahat ng klase)	1 bottle (330mL)	Doubles flare risk	Doubles	EXTREME
Gin / lambanog / brandy / whiskey	1 shot (45mL)	Blocks UA excretion	Blocks U	EXTREME
Coke / Pepsi / Royal (HFCS)	1 can (330mL)	+0.3–0.5 mg/dL UA	+0.3–0.5	EXTREME
Softdrink kahit anong klase	500mL bottle	HFCS → urate	HFCS →	VERY HIGH
Sweetened milk-tea / frappuccino	1 large (500mL)	Fructose + sugar	Fructose	HIGH
Tang / Zest-O / powdered juices	1 glass	Concentrated fructose	Concent	HIGH
Fruit smoothie (mango/banana)	1 cup	Natural fructose	Natural f	MODERATE
Wine (red or white)	1 small glass	Less than beer/spirits	Less tha	MODERATE
Sports drink (Gatorade, Pocari)	500mL bottle	Sugar raises urate	Sugar ra	MODERATE
Brewed coffee (black)	1–3 cups/day	Lowers UA long-term	Lowers U	PROTECTIVE
Plain water	2.5–3 L/day	Improves excretion	Improves	PROTECTIVE
Low-fat milk / plain yogurt	1 cup	Lowers UA (orotic acid)	Lowers U	PROTECTIVE
Calamansi juice (unsweetened)	1 glass	Vitamin C → mild uricosuric	Vitamin (uric	PROTECTIVE
Tart cherry juice (unsweetened)	1/2 cup	Anti-inflammatory	Anti-infla	PROTECTIVE

HIDDEN URIC ACID DRIVERS IN FILIPINO FOOD & DAILY LIFE

HFCS Softdrinks (biggest trigger):

Coke, Pepsi, Royal, Sprite, energy drinks, milk-tea, Tang. HFCS → fructose → direct UA synthesis. One 500 mL bottle raises UA by 0.3–0.5 mg/dL within hours.

Hidden seafood & bagoong:

Bagoong in kare-kare/pinakbet, patis, hibe (dried shrimp) in lugaw/palabok. Knorr/Maggi bouillon cubes = condensed meat/seafood extract (very high purine).

Sabaw (broth) trap:

Purines leach into broth of nilaga, sinigang, bulalo, bone broth. Eat the meat in small portions. SKIP THE BROTH entirely.

Medications that raise UA:

Thiazide diuretics (HCTZ in Co-Diovan) — #1 drug-induced cause. Loop diuretics (furosemide). Low-dose aspirin 81 mg. Cyclosporine/tacrolimus. Do NOT stop cardiac meds; ask about switching to ARB (mildly lowers UA).

Dehydration & lifestyle:

Heat, gym sessions, food poisoning: dehydration triggers flares within 24 h. Crash diets, ketogenic diet: ketones compete with UA excretion. Weekend binge (alcohol + food + dehydration) = classic Monday-morning flare.

MEDICATIONS FOR GOUT — FLARE TREATMENT VS. LONG-TERM PREVENTION

Medication	Purpose	CKD Notes	Local Name (PH)
Febuxostat 80mg OD	Long-term UA lowering (preferred in CKD)	No dose adjust until eGFR <15. Take DAILY, not only during flares.	Feburic, Uricfree, Atenurix
Allopurinol 100–300mg	Long-term UA lowering (alternative)	Dose-reduce in CKD. Check HLA-B*5801 first (severe skin reaction risk).	Generic allopurinol
Colchicine 0.5mg	Flare treatment + prophylaxis when starting ULT	Most effective within 12–24 h. Reduce dose in CKD. Avoid with clarithromycin.	Colcrys, generic colchicine
Prednisone / Methylpred	Acute flare (when NSAIDs contraindicated)	Preferred in CKD/elderly. Short course 5–7 days. Monitor BG in diabetics.	Generic prednisone
Naproxen / Indomethacin	Acute flare — NOT for CKD patients	AVOID in CKD, heart failure, peptic ulcer. Only if kidney function is preserved.	Generic NSAIDs

DO NOT start febuxostat/allopurinol during an active flare — wait until fully resolved (2–4 weeks). When starting ULT: prescribe colchicine 0.5mg OD for 3–6 months to prevent paradoxical flares.

WHAT TO DO DURING A GOUT ATTACK — 4-STEP PROTOCOL

1. **Act within 12–24 h:** Start anti-inflammatory medication IMMEDIATELY at first sign (colchicine or prednisone). Do NOT wait for the flare to "peak."
2. **Rest & ice:** Rest and elevate the joint. Ice wrapped in a towel, 20–30 min several times daily. Even bedsheet pressure hurts — use a pillow cradle.
3. **Drink 3 L of water today:** Flush uric acid through the kidneys. Avoid alcohol, HFCS drinks, sweet juices. Plain water, black coffee, or unsweetened calamansi juice.
4. **Do NOT stop febuxostat/allopurinol:** Stopping and restarting ULT causes UA fluctuations that prolong the flare and trigger additional attacks. Continue your maintenance dose throughout.

THE FILIPINO "FLARE DAY" PATTERN: Saturday: beer + sisig + chicharon → Sunday: kare-kare + bagoong + lechon + softdrinks + isaw → dehydrated sleep → **Monday 3AM: big toe on fire.**
THREE FIXES prevent 80% of these: stop softdrinks · stop beer · drink 2–3 L water on weekends.

Full guide with UA calculator, FEUA tool, FAQ, medication guide & flare protocol: williamriveromd.com/guides/gout-uric-acid